

WEIGHT LOSS

GI Side Effect Management

Grading, the stepwise management ladder, and the red flags that mean stop today

DOCUMENT WL-005	VERSION 1.0	EFFECTIVE January 2026	REVIEW CYCLE Annual
APPLIES TO All clinical staff	OWNER Medical Director	CLASSIFICATION Confidential	SUPERSEDES All prior versions

1 Purpose & Scope

Gastrointestinal effects are the main reason patients stop GLP-1 therapy, and most discontinuations are avoidable with structured management. This protocol grades severity, sets a stepwise response, and defines the findings that require the drug to be held and the patient evaluated the same day.

ASK, DO NOT WAIT

Patients frequently under-report nausea because they associate it with the treatment working, or fear the dose being reduced. Ask specifically at every contact and grade the answer.

2 Severity Grading

GRADE AT EVERY CONTACT

Grade	Presentation	Action
Mild	Occasional nausea, tolerating oral intake, normal activity, no dehydration.	Dietary counseling. Continue the current dose. Reassess at the scheduled visit.
Moderate	Persistent nausea, occasional vomiting, reduced oral intake, early dehydration signs, interfering with daily activity.	Hold escalation. Consider dose reduction. Antiemetic per standing order. Reassess within 7 days.
Severe	Intractable vomiting, unable to keep fluids down, orthostatic symptoms, severe abdominal pain, or any red flag below.	Stop the drug. Same-day medical evaluation. Do not manage by telephone.

3 Management Ladder

Step 1 — dietary and behavioural

- Smaller, more frequent meals; stop eating at the first sense of fullness.
- Reduce fatty, fried and highly spiced foods, which empty slowly and worsen symptoms.
- Separate fluids from meals; sip steadily through the day rather than with food.
- Eat slowly and remain upright for a period after eating.
- Prioritize protein even when appetite is low — see WL-007.
- Avoid alcohol, which worsens nausea and dehydration.

Step 2 — symptom-specific measures

Symptom	Approach
Nausea	Dietary measures first. Antiemetic per the medical director's standing order if inadequate. Reassess within a week.
Constipation	Fluid and fiber first, then a stool softener or osmotic laxative per standing order. Persistent constipation with distension and vomiting is a red flag, not a laxative problem.
Diarrhea	Assess hydration. Review other medications and dietary triggers. Persistent diarrhea warrants evaluation for another cause.
Reflux	Meal timing, upright posture, portion size. Acid suppression per standing order if persistent.
Early satiety with inadequate intake	This is a nutritional problem, not just a comfort problem. Address per WL-007 and consider dose reduction.

Step 3 — dose modification

- Hold escalation for any moderate symptom that is not settling.
- Reduce to the previously tolerated dose for persistent moderate symptoms, and remain there for at least the standard interval before reconsidering.
- Extend the interval at the current dose rather than reducing, where the prescriber judges that appropriate.
- Discontinue for severe symptoms, pending evaluation.
- Record the modification and the reasoning in every case.

4 Red Flags

HOLD THE DRUG AND ARRANGE SAME-DAY EVALUATION

Each of these represents a potentially serious complication rather than a tolerance issue.

Finding	Concern
Severe persistent abdominal pain, often radiating to the back, with vomiting	Acute pancreatitis.
Right upper quadrant pain, fever, jaundice, pain after fatty meals	Gallbladder disease — risk is increased during rapid weight loss.
Abdominal distension, absent bowel movements, vomiting, no flatus	Ileus or bowel obstruction.
Inability to keep any fluid down for an extended period, dizziness on standing, reduced urine output	Significant dehydration and acute kidney injury risk.
Vomiting blood, or black tarry stools	Gastrointestinal bleeding.
Shakiness, sweating, confusion — particularly on insulin or a sulfonylurea	Hypoglycemia. Check glucose; follow the emergency protocol.

DEHYDRATION IS THE QUIET ONE

Reduced intake plus vomiting or diarrhea, in a patient already eating very little, causes dehydration and acute kidney injury more often than the dramatic complications do. Assess hydration at every contact where symptoms are present, and have a low threshold for pausing therapy.

5 Documentation & Follow-Up

- ☐ Symptoms asked about specifically at every contact, and the grade recorded.
- ☐ Hydration and oral intake assessed whenever symptoms are present.
- ☐ Every intervention recorded, including dietary counseling given.
- ☐ Dose modifications recorded with reasoning.
- ☐ Red flag screen documented as negative where relevant — not merely omitted.
- ☐ Follow-up interval shortened whenever symptoms are moderate.
- ☐ Any adverse event reported to the manufacturer and to FDA MedWatch where applicable.
- ☐ Patients given written red-flag warning signs and after-hours contact instructions.

6 Quick Reference

QUICK REFERENCE CARD

Patient reports GI symptoms

1. **Grade it:** mild, moderate or severe.

2. **Screen the red flags** before anything else.

3. **Mild:** dietary measures, continue dose, reassess as scheduled.

4. **Moderate:** hold escalation, consider reduction, reassess within 7 days.

5. **Severe:** stop the drug, same-day evaluation. Not a phone call.

6. **Back pain + vomiting → pancreatitis until excluded.**

7. **Distension + no bowel movement + vomiting → obstruction, not constipation.**

8. **Assess hydration every time.** It is the most common real harm.

Print, laminate and post at the point of care

WL-005 GI Side Effect Management — MedSpa Standards v1.0 · January 2026

Page 3 of 4

7 Medical Director Authorization

This protocol is adopted as a standing order for the practice named below. By signing, the medical director confirms the protocol has been reviewed against current clinical guidance, applicable state scope-of-practice rules, and the training level of the staff authorized to perform it.

PRACTICE NAME	LOCATION / SITE
MEDICAL DIRECTOR — PRINTED NAME & CREDENTIALS	LICENSE NUMBER & STATE
SIGNATURE	DATE ADOPTED / NEXT REVIEW DATE

Template notice. This document is an editable template provided for educational and operational guidance. It is not legal advice, does not constitute a physician–patient relationship, and does not guarantee regulatory compliance or the outcome of any inspection. Drug doses, concentrations and device settings must be verified against current manufacturer labeling and prevailing clinical guidance before use. A licensed medical director must review, customize and sign this protocol before it is placed into service, and must confirm that every task assigned within it falls inside the scope of practice of the personnel performing it in your state.

© MedSpa Standards. Licensed for use within the purchasing practice only. Redistribution or resale is prohibited.